

Comment Submitted by Brandon In

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Brandon In respectfully submits the attached comment in response to the HHS Request for Information, *Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care* (FR Doc. 2025-23641).

I. Introduction

I have been developing software in healthcare for over a decade from:

1. Building robots for hospitals at InTouch Health (acquired by Teladoc Health),
2. Reducing physician burnout at One Medical,
3. Improving access to care at Optimize Health,
4. Pioneering permanent solutions for depression at Osmind, and
5. Reducing the cost of healthcare for all Americans by combating fraud, waste, and abuse at my own startup Claimformant.

While I support the goal of enhancing clinical productivity, adoption is currently constrained less by technical capability than by a mismatch between data infrastructure and clinical workflows controlled by a few dominant players.

Without a structural shift in how the government de-risks new technology and funds shared data infrastructure, clinical AI will remain a tool of legacy vendors rather than a transformative force for all American patients.

II. Executive Summary

The current healthcare technology ecosystem suffers from systemic failures that prevent true innovation from flourishing. Digitization through the HITECH Act of 2009 has inadvertently fostered vendor monopolies and high-cost regulatory hurdles, such as the prohibitive cost of HIPAA compliance, which stifle the entry of innovative startups.

To restore competition and improve health outcomes, HHS must move beyond strategy to infrastructure. We propose the creation of a government-backed, "YC-style" AI accelerator that provides a federal "stamp of approval," backed by a national insurance fund and legal safe harbors to de-risk startup adoption for large healthcare organizations.

Furthermore, we urge HHS to fund healthcare data exchanges as public utilities, mandate deterministic governance to ensure auditability, and fix the economic "efficiency penalty" that currently punishes providers for using AI to work faster.

III. The Innovation Crisis: Monopolies and Compliance Costs

The transition to digital records has failed to create a competitive market, leading instead to vendor opacity and lock-in where large legacy systems control fragmented data silos.

- **The Compliance Barrier:** The massive financial burden of HIPAA compliance and complex regulatory requirements serves as a primary barrier to private-sector innovation.
- **The Chilling Effect:** Small innovators fear investing in tools that may later be categorized as regulated medical devices, while institutional legal teams delay adoption for 12 months or more to avoid perceived liability.

IV. Proposal: A Government-Backed AI Accelerator and Safe Harbors

To create a level playing field, the government should establish a “Y Combinator-style” accelerator for healthcare AI startups. This program would act as a trust-bridge through:

- **Voluntary Certification:** An ONC-administered program to provide a “stamp of approval” validating safety, interoperability, and bias mitigation.
- **National AI Insurance Fund:** A government-backed fund, similar to the National Vaccine Injury Compensation Program, to compensate for rare cases of AI error, removing the massive liability insurance requirements that block small vendors.
- **Legal Safe Harbors:** Clear guidance providing legal safe harbors for providers who implement tools from these government-vetted startups.

V. Economic Alignment: Fixing the “Efficiency Penalty”

Adoption is throttled by the “Efficiency Penalty” in labor-based payment models (RVUs), which financially penalizes providers if an AI tool allows them to complete a task faster.

- **Tiered Reimbursement:** HHS should adopt a Tiered Complexity Model: Tier 1 for administrative software, Tier 2 for assistive pattern recognition, and Tier 3 for high-complexity predictive modeling.
- **AI-Guided Interpretation Codes:** CMS should introduce billing codes for “AI-Guided Clinical Interpretation” to reimburse the cognitive labor of a clinician verifying AI insights.

VI. Governance & Engineering: Determinism and Auditability

To bridge the trust gap, oversight must shift from statistical confidence to deterministic safeguards and audit-by-design.

- **Deterministic Governance:** High-risk clinical tools should be verifiable utilizing evaluation harnesses that allow changes in models, agent harnesses, and code changes to produce similar results.

- **Agent Trajectory Audit Trails:** Systems should generate agent trajectories for every decision, allowing third-party auditors to reconstruct and "replay" the process bit-for-bit after an adverse event with.

VII. Responses to Specific RFI Questions

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

Biggest Barriers to Private Sector Innovation and Adoption The primary barriers are systemic: unclear governance expectations, prolonged review cycles (12+ months), and fragmented data silos that rely on static, episodic inputs. Financially, the "efficiency penalty" in current RVU models penalizes providers for working faster, while high HIPAA compliance costs and liability uncertainty create a "chilling effect" for startups.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

CMS should introduce billing codes for "AI-Guided Clinical Interpretation" and amend 42 CFR § 410.78 to permit concurrent billing for remote monitoring when distinct data streams are used. Programmatically, a "YC-style" accelerator and a national AI insurance fund are essential to de-risk the market.

10. Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

HHS should prioritize research in high-potential tools including precision medicine platforms specifically with Neurotech. Generating more data from brain scans including fMRI, CT, EEG, EKG, and others allow us to guide precision therapy, diagnosis, and clinical guidance expanding access to care across all Americans especially understand rural areas and people of color.

VIII. Conclusion

By pulling the levers of regulation, reimbursement, and infrastructure to protect small innovators and enforce deterministic safety, HHS can break the current cycle of market concentration. We urge HHS to prioritize a government-backed accelerator and utility-style data funding to turn AI from a liability concern into a verified asset for all Americans.

Respectfully submitted,
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